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Mapping Inpatient Psychiatric Environments: Exploring the Intersection of Trauma, Recovery, and the Geography of Psychiatric Spaces

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Abstract :

This article explores the intersection of trauma, recovery, and the geography of inpatient psychiatric environments. It examines how modern understandings of trauma and recovery influence the physical and cultural landscapes of psychiatric institutions, particularly in Western contexts like the U.S. and the U.K. The paper argues that these environments both shape and are shaped by contemporary cultural scripts and medical practices. Through an analysis of historical trends, such as deinstitutionalization, and concepts like "carceral landscapes" and "permeable institutions," the study highlights the complex relationship between space, surveillance, and patient care. It also discusses "healing architecture," emphasizing how spatial design in psychiatric wards can impact recovery and the patient experience. The paper concludes that inpatient psychiatric environments are dynamic spaces where cultural, clinical, and architectural elements converge, influencing both the treatment of mental illness and the broader societal understanding of trauma and recovery.

Keywords : Trauma, Recovery, Deinstitutionalization

Introduction :

The definition of inpatient psychiatric settings, often known as institutions or units, varies depending on their specific context. As the notions of trauma and recovery have evolved and acquired new significance throughout history, the notion of inpatient psychiatric settings has also undergone a similar transformation. The architectural layout of these psychiatric settings has mirrored these transformations, as contemporary comprehension of trauma and rehabilitation has an impact on an institution's spatial orientation and Space. The geography of inpatient psychiatric settings encompasses the geographical location, architectural design patient population, staff composition, treatment modalities.

Each of these concepts is connected to contemporary knowledge of trauma and rehabilitation. The present essay aims to investigate the contribution of a study on the geography of inpatient psychiatric settings to the current study of trauma and recovery.

Cultural Scripts & Geography of Institutions :

Institutions, such as schools, and hospitals, reflect the geopolitical characteristics of their respective societies, and inpatient psychiatric settings are no different (1). Understanding of trauma recovery, which shape psychiatric

settings, is influenced by historical and cultural factors. Psychological reactions to trauma are intricate, going beyond the basic symptoms outlined in mental disorder categorization systems. The cultural context is of prime importance as distress is not encountered in isolation, so affecting social networks. Psychological trauma shapes developing patterns over time, which subsequently impacts culture (2). Clinical practitioners and therapists apply their own perspectives to the process of healing, focusing on the shared characteristics among patients and the specific aspects of their understanding of a condition. Culturally significant factors contribute to the formation of these filters, leading to culturally patterned manifestations of illness and Well-being. Patients and clinicians analyze and communicate information based on cultural frameworks of pain, significance, and desired outcomes, which differ among different cultures. Likewise, cultural paradigms of normality Functionality, encompassing emotions and relationships, varies among different cultures (2). The cultural context influences our encounter with trauma and the process of healing, and reciprocally, our journey of trauma and recovery influences our cultural environment. It is not a unidirectional effect, but a reciprocally influential relationship (2).

The present study will proceed to assert that the same assertion holds true for inpatient psychiatric care. Environments are influenced by current knowledge of trauma and recovery, and they also influence and inform these current knowledge. Although inpatient psychiatric settings have not traditionally Traditionally examined from a trauma-focused perspective, this analytical approach is undergoing development as the connection between trauma and mental illness is progressively confirmed (2). Currently accepted knowledge on trauma, The interdependence of mental illness and culture precludes any separate analysis, as they mutually influence each other. By virtue of the cultural scripts that influence institutions, inpatient psychiatric institutions worldwide assume various physical forms, represent diverse geographical locations, and embody distinct perspectives on trauma and rehabilitation. Over time, inpatient psychiatric institutions establish their own unique clinical-cultural milieu. Through my investigation of this paper, I discovered a common constraint in several publications: the application of research conducted in inpatient psychiatric settings in Western 'developed' countries, which have comparable periods of institutionalization and deinstitutionalization, to a global theory that was meant to be applicable to a wider context. Within the literature, it is common to overlook and presume that changes in psychiatric institutions in the Western world are universally applicable.

However, this assumption is unfounded. Regarding deinstitutionalization, it is worth noting that while it may have significant impacts in high-income countries, the widespread deinstitutionalization and closure of mental hospitals have not been the prevailing narrative in many low- and middle-income countries (LMICs). Historical background of Inpatient psychiatric settings in the USSR differ significantly from those in the U.S. and U.K. due to the deliberate use of psychiatry as a means of political torture and suppression (5). Consequently, research on the geographical distribution of inpatient psychiatric settings will exhibit significant variations among different regions. Soviet republics. The contention put forth is that the contemporary interpretations of trauma and recovery are influenced by the particular context and cultural background, hence affecting their interaction with inpatient psychiatric settings as well. Therefore, this paper will analyze the geographical distribution of inpatient psychiatric settings have experienced deinstitutionalization. Particular attention will be given to the United States and the United Kingdom, as several theories addressed in this paper are influenced by the medical history of those countries.

History of Inpatient Psychiatric Environments :

A significant portion of the documented history of institutionalization focuses on the United States and the United Kingdom. In the vanguard, both nations have a comparable historical trajectory of institutionalization and deinstitutionalization. Before the 17th century, individuals with mental illness received care through home-based services (7). The establishment of public and private hospitals in the United States during the early 18th century coincided with the development of psychiatric institutions. By the late 19th century, every state had constructed at least one public psychiatric institution. A total of 500,000 patients were admitted to institutions by the mid-20th century (7). However, this figure decreased as the principles of deinstitutionalization and community-based care gained widespread acceptance in the United Kingdom and the United States (9). Deinstitutionalization became a policy implemented in public hospitals during the civil rights movement, a period characterized by the assimilation of socially disadvantaged groups into the dominant society (10). Three pivotal parameters facilitated the shift of persons with profound mental illness from institutions to Community-based care: the perception of mental institutions as punitive and devoid of compassion, the optimism surrounding novel antipsychotic drugs as possible remedies, and the financial incentive to decrease expenses (10).

Carceral Landscapes :

The closure of psychiatric institutions in the U.S. and Western Europe is commonly regarded as a highly successful endeavor. Nevertheless, in the context of the United States, where the infrastructure of community-based care is lacking, the lack of adequate investment in care resulted in the effective replacement of psychiatric institutions with incarceration, so substituting one institution for another (11). In the initial stages of deinstitutionalization, during the 1960s and 1970s, researchers observed a rise in the number of inmates who had previously been admitted to a psychiatric institution (11). A comprehensive study completed in all 50 states revealed that the number of mentally ill incarcerated individuals increased by 77% (11) during deinstitutionalization efforts. Consistently, around 20% of those now imprisoned in the United States have a documented record of psychiatric disorders (12). As a consequence, prisons are transformed into a contemporary iteration of psychiatric institutions, yet they lack the purpose or capacity to provide inmates with assistance for their mental health issues. A mere 63% of persons with a prior record of mental illness avail themselves of mental health care during their imprisonment in state and federal prisons, and this percentage decreases to 45% for those now detained in local jails (12).

An editorial published in *Lancet Psychiatry* began by asking, "Have asylums really disappeared, or have they only undergone a fundamental change?" (13). The selection of the term "vanished," The terms "transformed" and "essence" prompt me to contemplate our tendency to perceive institutions as mere physical constructions, as the political, cultural, and legal systems that uphold them are often elusive or imperceptible (14). The prison system, in a similar manner, confines and inflicts harm upon vulnerable individuals, while simultaneously isolating them from society, so preventing their experiences from being acknowledged and valued (15). Psychiatric institutions and prisons are components of a 'carceral landscape', which refers to a collection of institutions such as prisons, asylums, hospitals, and workhouses that provide specific spaces for neglected or oppressed populations who have been forcibly 'exiled'. The landscapes of these institutions are characterised by large buildings, situated on vast and isolated grounds, with numerous rooms and corridors, and substantial resident populations. These landscapes are intended to enhance the mental and physical well-being of individuals, while simultaneously restraining, controlling, and treating their physical bodies. The visual and ideological expressions of this 'improvement' are determined by current conceptions of trauma and recovery.

Space and Surveillance :

The shift from psychiatric institutions to prisons occurred discreetly and without notice. In his book "Discipline and Punishment," Foucault argues that the mastery of visibility/invisibility bestows authority upon institutions, and for this authority to be exerted, it must be equipped with the means of Perpetual, comprehensive, all-encompassing monitoring, with the ability to render everything visible, as long as it could maintain its own invisibility" (14). Although Foucault is particularly addressing surveillance in correctional facilities, it is crucial to acknowledge the influence of surveillance in inpatient psychiatric settings. Video surveillance cameras and security personnel are employed to monitor inpatient units (16). Patients are mandated to occupy predominantly open and thus conspicuous areas. They are prohibited from entering their bedrooms between 9am and 4pm. However, there are designated spaces for staff to operate discreetly. Rooms and areas marked with "staff only" signs have limited entry (16). For his analysis, Foucault examines how the architectural evolution of the panopticon, a circular penitentiary with cells organized around a central watchtower, enabled inmates to experience intense surveillance. He states, "He is observed, but he lacks perception; he is a recipient of knowledge, never an active participant in communication" (14). Consequently, prisoners started self-regulating, as they were uncertain. In the digital era, surveillance has expanded to encompass various locations. Due to the presence of CCTV cameras, patients are aware that they are constantly being monitored and feared potential consequences. Being observed and documented, potentially resulting in self-regulation within psychiatric settings. As inpatient psychiatric settings have developed and transformed, their methods of monitoring have also advanced. It is crucial to examine the influence of modern conceptions of trauma and rehabilitation on these environments. An analysis of surveillance measures and how current surveillance strategies influence notions of trauma and recovery will be conducted throughout the paper.

Total and Permeable Institutions :

During the 1950s, Erving Goffman revolutionized our perception of the criminal environment by conducting ethnographic research in psychiatric institutions. His objective was to comprehend the social interactions within these institutions by fully immersing himself in the lives of the patients. As part of this study, he introduced the concept of a "total institution," which he defined as "a place of residence and work where a significant number of individuals, isolated from society for a considerable period

of time, collectively lead a closed, previously regulated cycle of life" (17). He portrayed psychiatric institutions as isolated systems apart from society, where patients received Within these total institutions, patients received custodial care and lived their lives exclusively within the hospital, with minimal external contact. Every aspect of their daily routines was rigorously controlled, and they Goffman elucidated the manner in which these patients encountered a state of identity deprivation due to physical and social mistreatment, undergoing a phenomenon known as "mortification of self" whereby their previous roles were systematically eliminated, leaving them with solely their institutional role (17). Psychiatric institutions were compared by Goffman to prisons, concentration camps, and monasteries, where patients encountered limited autonomy, social disapproval, and deprivation of social responsibilities. He observed that the sole distinction between a psychiatric institution and a prison was that the patients had not been previously incarcerated. Having been found guilty of a crime (6). However, in the context of the United States, where Goffman conducted his ethnographic research, mental illness would ultimately be rendered illegal. As the process of large-scale deinstitutionalization progressed in the United States, and community-based care was not prioritised.

The criminalization of mental illness resulted in the transfer of patients who would have otherwise required lifelong confinement in a psychiatric facility to a different type of correctional facility, namely a prison (18). Numerous factors have contributed to the criminalization of persons with mental illness in the United States. Primarily, policies such as "zero tolerance" policing, nuisance laws, and mandatory Sentencing for drug offenses has had a substantial impact (12). These policies frequently lead to a disproportionate targeting and incarceration of persons with mental illness. Furthermore, there is a prevailing belief that those with mental illness are intrinsically violent, which contributes to their stigmatization and higher probability of involvement with the criminal justice system (12). Moreover, the lack of a comprehensive mental health crisis response infrastructure exacerbates the situation, as the police are frequently called upon to assist individuals experiencing mental health crises. Engaging police officers, who are often untrained in mental health care, can Aggravate the circumstances and lead to either physical injury or imprisonment of the person (12). These elements together contribute to the disproportionate number of persons with mental illness in the criminal justice system. In Asylums (17), Goffman acknowledged certain permeable features of psychiatric institutions at the time but primarily highlighted their totalness, considering exceptions to be rare.

Goffman refers to the 'total institution' as an ideal for real world case studies to be referred to (17). In the ethnography "The permeable institution: An ethnographic study of three acute psychiatric wards in London"(16); Quirk argues that the metaphor of a total institution remains valuable, however the emphasis on totalness over permeability is outdated. Quirk offers a more relevant alternative ideal, the 'permeable institution" (16). This is a very clear example of how contemporary understandings of trauma and recovery shape our interpretation of in-patient psychiatric Ecosystems. Based on their ethnographic study, Quirk argues there is evidence of increasing permeability in today's inpatient psychiatric environments. Evidence of permeability includes the comparatively acute nature of treatment, with frequent changes due to short patient stays and high staff turnover. The study questions whether the difficulties of totalness would be possible for patients, as their inpatient treatment is temporary (16). Other signs of permeability include, patient's maintaining contact with the outside world, family and friends visiting, and staff working both inpatient and outpatient jobs (16). While there are obvious benefits to increased permeability, namely reduced risk of institutionalism, there are also negative consequences.

The accessibility of drugs and illicit substances infiltrating the wards, facilitated mainly by visitors, mirrors patterns observed in other supposedly closed institutions like prisons (16). Another example given in the ethnography was of a former patient who came back and threatened staff members. In reaction to the increased levels of permeability and the associated risks, Quirk observed a rise in surveillance within the wards. Closed-circuit television (CCTV) cameras are strategically positioned at every entry and exit point, including entry phone systems and security key-pad access. Access to these key-pads is restricted to authorized personnel (16). Security guards and designated nurses oversee the main entrance. Staff frequently exercise their own judgment in determining which visitors are permitted to enter the ward, often based on previous illicit encounters with visitors (16). However, as their discretion extends to innovative strategies for managing the consequences of the infiltration of local street culture into the ward (16), I contend that staff's Surveillance is influenced by the cultural scripts of the individuals involved. Quirk provides an instance where a nurse, prying into a phone conversation, intervened to prevent a patient from engaging in drug trafficking. Meanwhile, some nurses deliberately disregarded drug trafficking due to their profound fear of the repercussions for both themselves and the patient (16). Although the primary focus of surveillance appears to be monitoring the individuals and objects entering the ward, there is also surveillance conducted



within the ward to inhibit any attempts to escape (16). An effective strategy to reduce the likelihood of patients leaving before being approved for discharge is to place individual patients under "special observation" (16). A nurse is designated to remain within close proximity to the patient at all times. Special observation serves not only as a method of the management of permeability serves as a means to reduce the risk of self-harm. If these measures are considered insufficient in preventing "escape attempts," the staff may opt to lock the ward door or transfer the patient to a locked intensive care unit (16). The degree of control exerted by the staff over patients is determined by the distance between them, as demonstrated by the nurse maintaining a safe distance (16) from the patient. Similar themes of space and surveillance are evident when they lock doors, while increased permeability I am curious about the implications of the rise in staff time allocated to surveillance measures on care and recovery. Both the ideals of total and permeable institutions reflect the surveillance methodologies described by Foucault in prisons. This indicates that these two concepts will continue to intersect. Quirk suggests that our understanding is of a spectrum of institutional permeability, with total and permeable institutions at opposite ends of the spectrum (16). The study includes a visual representation (below) of this spectrum.

permeability in institutions, characterized by total and permeable conditions on both ends. Many of the parameters used to quantify permeability are geographical, such as "geographical location" and "openness of facilities". The importance of space in defining permeability highlights the impact of geographical concepts on current views of trauma and recovery. Considering conceptualization along a spectrum reflects current understandings of trauma and recovery. Different cultures have varying degrees of comfort with spectrum thinking. Societies like the U.S. and the U.K., which have experienced deinstitutionalization, are increasingly recognizing the existence of trauma and recovery frameworks. Across a spectrum. Nevertheless, this has not consistently been the situation.

Healing Architecture and Recovery :

Contemporary conceptualizations of trauma and recovery have influenced the field of inpatient psychiatry. The relationship between architecture and healing has been established in various settings. This field of research is known as "healing architecture," which investigates how specific characteristics of the constructed environment affect healing. Environmental factors, such as lighting and atmosphere, access to and/or views of green spaces, accessibility and openness, positively influence patient experience, wellbeing, and recovery (19). In clinical settings, the physical environment influences medical encounters, interpersonal interactions, and socio-technical practices. Similarly, healing architecture establishes a space that promotes healing. The relationship between recovery and space is explored through the therapeutic effects of architecture, known as spatial orderings. This concept acknowledges that the physical layout of a ward affects psychiatric practices by organizing the distribution of individuals, assigning roles, deciding the location of specific activities, and defining how these activities are considered appropriate within their designated spaces. Spatial ordering provides a means to investigate the process of healing. The study "Healing architecture and psychiatric practice: (re)ordering work and space in an in-patient ward in Denmark" (19) demonstrated that as the ward became more open and transparent, staff were compelled to enact spatial orderings as a result of the mediations facilitated by architectures in mediating experiences, practices, activities, and encounters...and how these mediations re-order such spaces, creating new, albeit transient, 'geographies of care,' The implementation of safety and control measures to effectively manage patients on a daily basis (19) aligns with Quirk's research findings on permeability and surveillance (16). During an ethnographic phase of healing architecture

research in Denmark, it was observed that the staff utilized space to evaluate, regulate, and supervise patients. In the instance of a specific patient's behavior (referred to as patient A), the chief physician reported expressing dissatisfaction with "the" In response to the head-nurse's conviction that it should not be tolerated, she stated, "he uses a lot of square meters." The staff were quantifying Patient A's inappropriate behavior by the amount of space they were occupying in the ward. It was unclear whether Patient A was occupying this space literally or metaphorically (19). As the staff deliberated on how to separate Patient A from the others, they started exploring the possibility of relocating Patient A to the seclusion room. Nevertheless, the difficulty lies in the fact that Patient M is presently confined to the seclusion room and there is ambiguity regarding the possible relocation of Patient M. The availability of rooms is restricted, and the small number of available rooms are The available options are either in close proximity to the exit or not visible to the staff. Both of these options are dismissed due to Patient M's repeated attempts to escape, rendering the room near the exit ineffective. Additionally, Patient M is experiencing suicidal ideation, which necessitates constant visibility to the staff. As the staff deliberates on the location for Patient M's relocation, one can observe the manifestation of spatial ordering within the institution's geography.

These acts of social ordering exemplify current perspectives on trauma and therapeutic processes. The spatial arrangement and implementation of isolation observed in inpatient psychiatric settings are indicative of In recent times, the field of trauma and psychiatry has developed, leading societies like the U.S. and U.K. to embrace cultural frameworks related to trauma that have been present in other cultures for a long time. This includes the recognition that trauma and It is my contention that subsequent posttraumatic distress does not arise in isolation from social interactions. Others may also experience trauma, and the effects on one individual may resonate throughout the social milieu. The cultural recognition that trauma does not occur in isolation ironically influences the staff's choice to isolate patients. The staff finds it necessary to isolate patients not only to safeguard them from each other but also to maintain control over the ward (19). They acknowledge that the distress of one patient can affect the entire ward. Concurrently, the staff closely monitors the patients through continuous surveillance, in order to protect them from both themselves and others. Nevertheless, if we acknowledge that trauma does not occur in isolation, should we not presume that the same applies to the process of recovery? This gives rise to one argument in favor of deinstitutionalization and heightened A fundamental principle of community-based

care is that patients cannot achieve recovery without being integrated into their community (6). Isolation has been extensively studied in hospital wards as a means of facilitating recovery. From the viewpoint of patients, periods of solitude were also found to be linked to recovery (6). Another ethnographic study, titled "Exploring In Patients Experiences of Healing and Healing Spaces," further supports this notion. The study conducted a mixed methods approach and interviewed acute hospitalization patients. The findings revealed that the presence of alone time was linked to the process of recovery. Patients attributed their own room, low noise environments, and a tranquil atmosphere to a healing architecture. However, patients did not use the term isolation when discussing healing and recovery, whereas staff practitioners did.

Patient Perspective and Involvement :

One further developing element of modern comprehension of trauma and recovery is the viewpoints of the patients. The field is increasingly acknowledging the significance of patients in shaping definitions of trauma and recovery, as well as the implementation of these concepts. When interviewed (20), patients identified seven design strategies that facilitated healing. These strategies encompassed single rooms, serene artwork, windows facing east, plants, and acoustic elements. Various factors, classified as "healing experience enhancers," were identified in response to inquiries about the inpatient psychiatric environment. These factors encompassed physical characteristics, positive emotions and comfort, sense of belonging, presence of family members, natural surroundings, spiritual and religious importance, and the quality of care provided by hospital staff. From the patient's standpoint, the notion of a healing space is influenced by both physical designs and architecture, as well as relationships, emotions, beliefs, and culture. The researchers identified multiple themes in this regard. Multiple environments coexisted concurrently within the healing environment, encompassing the physical, internal, behavioral, and external aspects. In the study, three main themes were identified as positive factors that influence a healing experience: receiving care, feeling comfortable, and experiencing a familiar or home-like environment. These findings indicate that patients view their inpatient healing experience as a supportive environment. Throughout the study, patients made highly detailed observations of the physical environment of the hospital, including the seating arrangements for visitors and the overall space surrounding their beds. The researchers deliberately choose to describe these observations using broad terminology, such as a

"supported environment," in order to ensure that the findings can be applied to various settings. While it is logical that a patient's path to recovery would facilitate a supportive environment, it is important to consider the implications of this in different contexts. How does a culturally specific understanding of trauma and recovery influence our perception of a supported environment? Additionally, how does a patient's mental health diagnosis or symptoms impact their perception of a mental health diagnosis? Throughout the study, patients consistently mention the notion of home and the sense of being at home while in the hospital as a means of healing. However, Palmer's research in "Fractal Heterotopia and the Affective Space of Psychosis" (21) revealed that patients suffering from psychosis and/or paranoia may be influenced by inpatient institutions attempting to replicate a homely atmosphere. The participants encountered familiar patterns resembling a sheep in wolves, which caused them to feel uneasy, as if the psychiatric setting was attempting to deceive them (21). Active participation of patients in research and practice is of utmost significance in the context of An examination of the correlation between current conceptions of trauma and recovery, and the context of inpatient psychiatric settings. Within any setting, including inpatient psychiatric settings, there are several cultural scripts occurring concurrently. Similar to the presence of several environmental settings within the healing environment (20), there are also several cultures that exist. There are several cultural factors that influence the institution, including the culture of the country or community where it is located, the culture of the community from which each patient originates, the culture of the institution itself, and potentially the cultural symbols associated with particular diagnoses and diseases. The context plays a paramount role in the study of the geography of trauma and recovery. This can enhance the challenge for research to attain replication.

Conclusion :

Further assessment is necessary to explore the correlation between current knowledge of trauma and recovery and inpatient psychiatric settings. Empirical investigations in this field illuminate the reciprocal relationship between the definitions of trauma and recovery and our institutions. The development and cultural contexts of psychiatric institutions have been intricately connected to the evolution of ideas concerning trauma and recovery throughout history. Through the transition from total institutions characterized by stringent control to the rise of more open environments, the geographical distribution of psychiatric care has mirrored shifts in society perspectives on mental health. Cultural beliefs, clinician perspectives, and patient

experiences collectively influence the configurations of inpatient psychiatric settings, which function as miniature representations of wider society attitudes towards mental illness, monitoring, and recovery. Finally, examining the geographical aspects of inpatient psychiatric settings offers a comprehensive perspective to investigate current conceptions of trauma and rehabilitation. Through an analysis of the geographical, cultural, and social aspects of psychiatric patient care, we can enhance our methodologies to The provision of assistance to persons in their journey towards healing and recuperation.

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References :

1. Philo C, Parr H. Institutional geographies: introductory remarks. *Geoforum* [Internet]. 2000 Nov 1 [cited 2024 Apr 7];31(4):513–21. Available from: <https://www.sciencedirect.com/science/article/pii/S001671850000018X>
2. Chentsova-Dutton Y, Maercker A. Cultural scripts of traumatic stress: outline, illustrations, and research opportunities. *Front Psychol* [Internet]. 2019 Nov 15 [cited 2024 Apr 7];10. Available from: <https://www.frontiersin.org/journals/psychology/articles/10.3389/fpsyg.2019.02528/full>
3. What is deinstitutionalization? [Internet]. [cited 2024 Apr 7]. Available from: <https://www.opensocietyfoundation.s.org/explainers/what-deinstitutionalization>
4. Cohen A, Minas H. Global mental health and psychiatric institutions in the 21st century. *Epidemiol Psychiatr Sci* [Internet]. 2016 Sep 19 [cited 2024 Apr 7];26(1):4–9. Available from: <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6998654/>
5. Alexéyeff S. Abuse of psychiatry as a tool for political repression in the Soviet Union. *Med J Aust*. 1976 Jan 31;1(5):122–3.
6. Chow WS, Priebe S. Understanding psychiatric institutionalization: a conceptual review. *BMC Psychiatry* [Internet]. 2013 Jun 18 [cited 2024 Apr 7];13:169. Available from: <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC3702490/>
7. Diseases of the mind [Internet]. [cited 2024 Apr 7]. Available from: <https://www.nlm.nih.gov/hmd/topics/diseases-of-mind/timeline.html>
8. History of psychiatric hospitals [Internet]. [cited 2024 Apr 7]. Available from: <https://www.nursing.upenn.edu/nh/hc/nurses-institutions-caring/history-of-psychiatric-hospitals/>
9. Lolas F. Mental illness and institutionalization. In: ten Have H, editor. *Encyclopedia of Global Bioethics* [Internet]. Cham: Springer International Publishing; 2016 [cited 2024 Apr 7]. p. 1888–92. Available from: https://doi.org/10.1007/978-3-319-09483-0_291
10. Yohanna D. Deinstitutionalization of people with mental illness: causes and consequences. *AMA Journal of Ethics* [Internet]. 2013 Oct 1 [cited 2024 Apr 7];15(10):886–91. Available from: <https://journalofethics.ama-assn.org/article/deinstitutionalization-people-mental-illness-causes-and-consequences/2013-10>
11. Siller S. Deinstitutionalization, Mental Health, and Criminal Populations: How the Process of Deinstitutionalization Affected Current Incarceration Rates of the Mentally Ill [Internet]. 2017.
12. National Alliance on Mental Illness (NAMI) [Internet]. NAMI; Criminalization of People with Mental Illness. Available from: <https://www.nami.org/Advocacy/Policy-priorities/Stopping-Harmful-Practices/Criminalization-of-People-with-Mental-Illness>
13. The Lancet Psychiatry null. The land of the free. *Lancet Psychiatry*. 2015 May;2(5):363.
14. Foucault, Michel. "Discipline and punish." A. Sheridan, Tr., Paris, FR, Gallimard (1975).
15. Cohen A, Minas H. Global mental health and psychiatric institutions in the 21st century. *Epidemiol Psychiatr Sci* [Internet]. 2016 Sep 19 [cited 2024 Apr 8];26(1):4–9. Available from: <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6998654/>
16. Quirk A, Lelliott P, Seale C. The permeable institution: An ethnographic study of three acute psychiatric wards in London. *Social Science & Medicine* [Internet]. 2006 Oct 1 [cited 2024 Apr 8];63(8):2105–17. Available from: <https://www.sciencedirect.com/science/article/pii/S0277953606002693>
17. Goffman E. On the characteristics of total institutions. *Symposium on preventive and social psychiatry*. Vol. 4384. 1961.
18. Dvoskin JA, Knoll JL, Silva M. A brief history of the criminalization of mental illness. *CNS Spectr*. 2020 Oct;25(5):638–50.
19. Simonsen TP, Duff C. Healing architecture and psychiatric practice: (Re)ordering work and space in an inpatient ward in Denmark. *Sociology Health & Illness* [Internet]. 2020 Feb [cited 2024 Apr 8];42(2):379–92. Available from: <https://onlinelibrary.wiley.com/doi/10.1111/1467-9566.13011>
20. MacAllister L, Bellanti D, Sakallaris BR. Exploring inpatients' experiences of healing and healing spaces: a mixed methods study. *J Patient Exp*. 2016 Dec;3(4):119–30.
21. Palmer, Laura. "Fractal Heterotopia and the Affective Space of Psychosis." *London Journal of Critical Thought*. 63.8 (2006): 2105-17.